

Sample Hospital — Cardiology

4200 Lakeshore Drive, Suite 310, Madison, WI 53703

Phone: (608) 555-0142 | Fax: (608) 555-0143

OUTPATIENT PROGRESS NOTE

Patient: Doe, Anna B.	DOB: 03/15/1958	MRN: SH-2024-088451
Visit Date: 01/19/2025	Visit Type: Follow-up, Post-Discharge	Provider: Dr. Jane Smith, MD, FACC

SUBJECTIVE

Mrs. Doe presents for 1-week post-discharge follow-up after hospitalization for acute decompensated heart failure. She reports significant improvement in her breathing since discharge. She is able to walk to the mailbox and back (approximately 200 feet) without stopping, compared to being unable to walk 50 feet on admission. She denies orthopnea, PND, or chest pain. Reports mild ankle swelling at end of day that resolves with elevation. Daily weights have been stable at 168-170 lbs (discharge weight 169 lbs, admission weight 177 lbs). She is following the sodium-restricted diet and fluid restriction 'mostly' — admits to one episode of eating restaurant soup yesterday. Sleeping flat with one pillow, no nighttime awakenings. Reports mild dizziness when standing quickly but no syncope.

OBJECTIVE

Vitals: BP 108/68 (sitting), 102/62 (standing), HR 72 regular, RR 16, SpO2 97% RA, Wt 169.4 lbs, Temp 97.8°F

General: Alert, oriented, in no acute distress. Well-appearing.

CV: Regular rate and rhythm. S1/S2 normal. Grade II/VI systolic murmur at apex (unchanged). JVP ~8 cm (improved from 14 cm on admission). No S3 gallop.

Lungs: Clear to auscultation bilaterally. No crackles, wheezes, or rhonchi.

Extremities: Trace bilateral pedal edema (1+). Warm, well-perfused. No calf tenderness.

Labs (01/15/2025): BNP 284 pg/mL (↓ from 1847), Cr 1.4, K 3.8, Na 140

ASSESSMENT

- Chronic systolic heart failure, recent acute decompensation — IMPROVING.** Excellent diuretic response. Euvolemic to mildly hypervolemic. BNP trending appropriately. Functional status markedly improved.
- Type 2 diabetes mellitus — SUBOPTIMAL CONTROL.** HbA1c 8.4% on admission. Will need intensification of diabetic regimen. Metformin alone insufficient.
- Orthostatic hypotension — MILD.** Likely volume-related. Will monitor; hold off on up-titrating carvedilol for now.
- CKD Stage III — STABLE.** Creatinine at baseline.

PLAN

- Continue furosemide 60mg BID. Recheck BMP in 1 week. Target weight 165-168 lbs.
- Add empagliflozin 10mg daily — dual benefit for HF and diabetes per EMPEROR-Reduced. Discussed with patient including risk of UTI and need for hydration monitoring.
- Defer carvedilol up-titration until orthostasis resolves. Recheck standing BP next visit.
- Cardiac rehab authorization approved (36 sessions). First session scheduled 01/22/2025.
- Reinforce dietary compliance. Referred to nutritionist (appt 01/24/2025).
- Follow-up in 2 weeks (02/02/2025) or sooner if weight increases >3 lbs/day or symptoms worsen.
- Continue all other home medications unchanged.

Jane Smith, MD, FACC

Board Certified: Cardiovascular Disease, Internal Medicine

Electronically signed 01/19/2025 11:42 CST

Time spent: 35 minutes (>50% counseling/coordination)

SYNTHETIC DATA — FOR DEMONSTRATION PURPOSES ONLY — NO REAL PATIENT INFORMATION